Reg. No. RMEE2552994
PURE PATH LAB
Add: Infront of Cantabil, Beside Distt. Hospital-Gonda
7703021000, 7897319900
www.purepathlab.in Gmail: help@purepathlab.in
Dr. Abha Mishra
M.B.B.S (M.D. PATH)
Reg. No-MP-8779
PURE PATH LAB
Palce of Accuracy & Your Satisfaction

Mr. Santosh Verma
Age / Sex : 53 YRS / Male
Reg. no. : 6959

Registered on : -
Collected on : -
Received on : -
Reported on : -

HAEMATOLOGY

TEST		VALUE	UNIT	REFERENCE
HEMOGLOBIN	**L**	**10.5**	**g/dl**	**13 - 17**
TOTAL LEUKOCYTE COUNT	**L**	**4300**	**cumm**	**4,800 - 10,800**
NEUTROPHILS		64	%	40 - 80
LYMPHOCYTE		29	%	20 - 40
EOSINOPHILS		05	%	1-6
MONOCYTES		02	%	2 - 10
BASOPHILS		00	%	<2
PLATELET COUNT		1.76	lakhs/cumm	1.5 - 4.1
TOTAL RBC COUNT	**L**	**2.79**	**million/cumm**	**4.5 - 5.5**
HEMATOCRIT VALUE, HCT	**L**	**28.5**	**%**	**40 - 50**
MEAN CORPUSCULAR VOLUME, MCV	**H**	**102.2**	**fL**	**83 - 101**
MEAN CELL HAEMOGLOBIN, MCH	**H**	**37.6**	**Pg**	**27 - 32**
MEAN CELL HAEMOGLOBIN CON, MCHC	**H**	**36.8**	**g/dl**	**31.5 - 34.5**
MEAN PLATELET VOLUME, MPV		11.1	fL	6.5 - 12
R.D.W. - SD	**H**	**57.1**	**fL**	**39 - 46**
R.D.W. - CV	**H**	**16.3**	**%**	**11.6 - 14**
P-LCR		34.6	19.7	- 42.4
P.D.W.	**H**	**16.8**	**fL**	**9.6 - 15.2**

Possible causes of abnormal parameters

	High	Low
RBC, Hb, or HCT	Dehydration, polycythemia, shock, chronic hypoxia	Anemia, thalassemia, and other hemoglobinopathies
MCV	Macrocytic anemia, liver disease	Microcytic anemia
WBC	Acute stress, infection, malignancies	Sepsis, marrow hypoplasia
Platelets	Risk of thrombosis	Risk of bleeding

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SEROLOGY & IMMUNOLOGY

TEST	VALUE	UNIT	REFERENCE
HIV - 1	NEGATIVE		
HIV - 2	NEGATIVE		
HEPATITIS C VIRUS, HCV	NEGATIVE		
HBSAG	NEGATIVE		

HbsAg

HbsAg is the first serologic marker appearing in the serum 6-16 weeks following hepatitis B viral infection. In typical HBV infection, HBsAg will be detected 2-4 weeks before the liver enzyme levels (ALT) become abnormal and 3-5 weeks before the patient develops jaundice. In acute cases HbsAg usually disappears 1-2 months after the onset of symptoms. Persistence of HbsAg for more than 6 months indicates development of either a chronic carrier state or chronic liver disease. The presence of HbsAg is frequently associated with infectivity. HbsAg when accompanied by Hepatitis Be antigen and/or hepatitis B viral DNA almost always indicates infectivity.

Anti HCV card test interpretation

Reactive test result indicates presence of Hepatitis C virus infection. Active infection to be confirmed by HCV RNA PCR test. It cannot differentiate between the stages of Hepatitis C viral infection nor used to monitor the efficacy of treatment. Non-Reactive test result indicates Hepatitis C virus infection is unlikely.

HIV Interpretation

A negative result implies that no Anti HIV-1 & HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been tested during the “WINDOW PHASE” (before the development of detectable levels of antibodies). A positive result suggests the possibilities of HIV-I and / or HIV-II infection. However these results must be verified by a confirmatory test (IFA / WESTERN BLOT I-II) before pronouncing the patient positive for HIV-1 and / or HIV-2 infection. All reactive samples should be confirmed by using HIV Western Blot/PCR.

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BIOCHEMISTRY

TEST		VALUE	UNIT	REFERENCE
RANDOM BLOOD SUGAR		112.8	mg/dl	70 - 140
SERUM UREA	**H**	**75.84**	**mg/dl**	**18 - 55**
SERUM CREATININE	**H**	**4.71**	**mg/dl**	**0.72 - 1.43**
SERUM PHOSPHORUS	**H**	**4.55**	**mg/dl**	**2.5 - 4.5**
SERUM URIC ACID		6.87	mg/dl	3.5 - 7.2
SERUM SODIUM		136.1	mmol/L	136 - 146
SERUM POTASSIUM		3.52	mmol/L	3.5 - 5.1
ICALCIUM		1.21	mmol/l	1.13 - 1.33

Creatinine is a nitrogenous waste product formed in muscle from creatine phosphate. Endogenous production of creatinine is proportional to muscle mass and body weight.

Causes of Increased Serum Creatinine Level	Causes of Decreased Serum Creatinine Level
1. Pre-renal, renal, and post-renal azotemia	1. Pregnancy
2. Large amount of dietary meat	2. Increasing age (reduction in muscle mass)
3. Active acromegaly and gigantism	

~~~ End of report ~~~
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